HMI Client Contact Hours (CCH) Log

Student: Jithendran Sellamuthu
AHA #: 007913
Client Initials: T.M.
Session #: 4
Date: November 6, 2026
Duration: 90 minutes
Modality: Zoom (Virtual)
Fee: Pro Bono (CSP)

1. Presenting Issue

T.M. is a 22-year-old student at Appalachian State University in North Carolina, referred through the HMI Community Service Program (CSP). He presents with anxiety, hypervigilance, trust difficulties, and self-esteem challenges rooted in childhood trauma. Root wound refined across Sessions 1-3 as abandonment and alienation: mother imprisoned (methamphetamine), father incarcerated (heroin), both absent during early childhood. Foster situations with neglect. Grandfather provided stability but was emotionally unavailable. Three uncles served as primary caregivers and were verbally and physically abusive. This session identified a critical new dimension: the uncle was a contradictory figure — both supportive/admired and physically frightening — creating a fused emotional response (fear + admiration) that now activates in any close social dynamic. The client reported that prior session reframing work (protector energy, helper narrative) had faded after approximately 1.5 weeks, feeling "forced and numbing" rather than genuine. Suggestibility: 31% Physical / 69% Emotional.

2. What preparation did you do for this session?

- Reviewed Session 3 notes, client's MKP journal (Oct 28), and clinical analysis of the 88/12 split (why behavioral tools cannot reach subconscious beliefs)
- Prepared a 22-phase enriched inner child readout integrating techniques from four sources: Kappasinian (Melton Room Technique, Machenberg wound model, Kappas reprogram-not-story, Nasir memory reconsolidation), HypnoticWorld (FearOfAbandonment affect bridge + mist metaphor + life path walk, InnerChild reparenting + "knock on door" post-hypnotic, SelfCriticism voice identification + screen replacement), Gemini AI Mode 3-Phase Model (Observer Canvas desensitization, Secondary Gain Acknowledgment, Mesh Filter PHA), and original TM-002 techniques (Flip Theory, tribe circle, protector energy, evidence chain)
- Prepared Observer Canvas desensitization with IMS (Ideomotor Signaling) gating: dominant finger for processing, desensitization confirmed below 2/10 before proceeding to inner child contact
- Prepared Secondary Gain Acknowledgment: validating the protective system before asking it to stand down
- Prepared Mesh Filter PHA (LEFT hand anchor for Tuesday group: selective barrier allowing connection in, dissolving old static)
- Produced "The Room You Build" — a 22-minute immersive guided visualization movie (14 scenes, ElevenLabs Sophia Empathetic narration, Flux/GPT-4o keyframes, FFmpeg Ken Burns assembly) for between-session practice
- Reviewed 13 HypnoticWorld scripts (FearOfAbandonment, InnerChild, SelfCriticism, FearOfRejection, Forgiveness, OvercomeShame, OvercomeGuilt, DevelopSelfCompassion, CopingWithRejection, FearOfBeingAlone, CircleOfConfidence) for technique extraction and adaptation
- Reviewed Gemini AI Mode conversation on sequencing desensitization, circle therapy, and inner child work for this client's specific presentation

3. What hypnotic techniques and suggestions did you use in this session?

Full hypnotherapy session with trance induction, deepening, and inner child reparenting. This is the first session in the case where sustained trance work was delivered (S1 = EFT/assessment, S2 = first trance with tribe circle, S3 = no trance/psychoeducation). The following techniques were employed:

- **Emotional-lane Induction:** Breathing lead-in (3 cycles), hand-on-chest affirmations ("I am great," "I am a wonderful person," "Everybody genuinely likes me," "I am being loved"), eyes-open visual anchor ("happy face" — identity as helper/protector), permissive eye closure, "Deep Sleep" PHS keyword. Consistent with S2 induction for repetition = association.
- **Deepening:** 5-0 countdown, 10-step staircase descending into the subconscious, progressive relaxation crown-to-toe with explicit instruction to release externally imposed burdens (insufficiency, incompetence, worthlessness framed as not intrinsic).
- **Observer Canvas Desensitization:** Client guided to a private screening room where released negative feelings took visible shape on a large screen. Instructed to drain color (vivid emotions → fading B&W photographs → pixelated meaningless symbols). Externalization technique creating safe observational distance.
- **Somatic Identification + Affect Bridge:** Client located the physical body location of the Tuesday group abandonment feeling. That bodily sensation was used as a thread/compass to trace the feeling back to its childhood origin point. Psychosomatic physiology explained: belief is stored in body tissue, not mind — why cognitive tools catch but cannot erase.
- **Mist Metaphor + Inner Child Meeting:** Adult self guided through mist of the past to encounter younger self. Subconscious trusted to identify the correct age without conscious selection. Mist parted by adult's presence (not force). Two core false beliefs identified in the child: "I am not worth being here for" (abandonment → unworthiness) and "I must be causing the anger" (guilt → self-blame).

- **Inner Child Reparenting:** Adult self corrected the false beliefs directly to the child: events were not the child's fault; the child was pure and innocent; guilt and shame are not his to carry. Agency language used — client discovered the words, therapist did not script them.
- **Secondary Gain Acknowledgment + Defensive Strategy Release:** Child's withdrawal, distrust, and isolation reframed as rational survival mechanism (firewall/watchtower). Adult acknowledged and thanked the protective part. Subconscious asked to signal readiness for the adult to take over — confirmed by involuntary right index finger lift (IMS).
- **Safe Room Construction + Integration:** Client built a safe room (walls, light, floor, temperature, sounds — all client-designed). Child and adult brought together inside. Guided to embrace, hold hands, and fully integrate. Client nodded to confirm integration felt complete. Deep breath carried integrated state into every cell.
- **Count Out:** Ascent via staircase, 0-5 count. Subconscious door explicitly closed to prevent re-entry of old doubts. Client returned to full waking awareness.

4. What were the highlights of this session?

- **Uncle identified as root trigger:** The critical new clinical data — the uncle was a contradictory figure (supportive AND frightening), creating a fused emotional response that now maps onto all close social dynamics. This is more specific than the general "abandonment + alienation" formulation from S3 and provides a precise target for cord cutting/realignment in S5.
- **Reframing fatigue validated:** Client reported the protector energy/helper reframe from S2 lasted ~1.5 weeks then felt "forced and numbing." This validates the clinical analysis that behavioral tools cannot reach somatically stored beliefs — and that trance-based inner child work was correctly indicated.
- **IMS confirmation:** The involuntary right index finger lift when the subconscious agreed to let the adult take over the protective role is the strongest evidence of trance engagement across all 4 sessions. The signal was unprompted and involuntary.
- **Integration confirmed:** Client nodded during the embrace/integration phase and reported feeling "more complete" immediately after emergence. This is the target outcome of the inner child protocol.
- **Client preference respected:** Client explicitly requested that direct memory-retrieval work be deferred — noting that revisiting traumatic memories in past therapy worsened anxiety for days. The inner child approach (bringing the child to a safe imaginal space rather than re-exposing to trauma) aligned with both the client's preference and the therapist's planned approach.
- **Cord cutting/realignment previewed:** Client was educated on both techniques for future sessions. Both deferred pending evaluation of how inner child integration settles.

5. Did you discuss this session with a supervisor and if so what guidance did you receive?

Prior to this session, I reviewed case conference guidance from four HMI instructors and conducted an extended clinical planning session using Gemini AI Mode:

- **Lisa Machenberg:** Two wound-specific hypnotic intentions delivered: abandonment ("you were always worth staying for") and shame/blame ("the anger was their dysfunction, not your fault"). Same trigger → new behavior → superior result framework applied to group dynamics.
- **John Melton:** Room Technique executed as designed: NEW room (not childhood place), adult meets child from current perspective, agency language (client discovers words). Integration was behavioral (nod confirmed) not compulsory. Recommendation to watch for intellectualization was applied — client engaged experientially rather than analytically.
- **George Kappas:** Addressed the PROGRAMMING (two specific false beliefs) not the story (childhood narrative). Moved efficiently from check-in to trance work. Did not dwell in narrative processing.
- **Saman Nasir:** Memory reconsolidation executed: childhood memory accessed in trance, adult perspective attached, re-filed with updated meaning. Somatic identification (body location of feeling) used as affect bridge — confirmed that belief is stored in body tissue, not prefrontal cortex.
- **Gemini AI Mode consultation:** 3-Phase model (Desensitize → Circle Therapy Alignment → Inner Child Integration) applied. Observer Canvas desensitization with IMS gating was deployed as Phase 1. Secondary Gain Acknowledgment validated the protective system before requesting stand-down. The "Trojan Horse" group protocol and Mesh Filter PHA were discussed but Mesh Filter was not installed in this session (deferred to S5 to avoid overloading).

6. What do you plan to do for the next session?

- **Receive client feedback:** How does the integration feel in real-life social situations (specifically Tuesday MKP group)? Is the wall thinner? Does the anxiety feel different? Do compliments land differently?
- **Cord cutting or cord realignment:** Depending on feedback, address the uncle relationship directly. Cord cutting if full emotional separation is appropriate (mutual release with respect). Cord realignment if the relationship has ongoing value (healthy boundaries + forgiveness while maintaining connection).
- **Mesh Filter PHA installation:** LEFT hand anchor for group settings (deferred from S4 to prevent overloading). Selective barrier: connection in, old static out.
- **Circle Therapy:** If cord work is not needed or is completed quickly, proceed to systematic desensitization of group triggers (4 confrontation cycles in trance).
- **Self-hypnosis teaching:** Begin formal self-hypnosis instruction if integration has stabilized. Client already performing informal pre-sleep trance (homework).
- **Monitor:** Reframing durability (does it hold longer than 1.5 weeks post-S4?), nightmare pattern, anchor usage, group behavior changes.